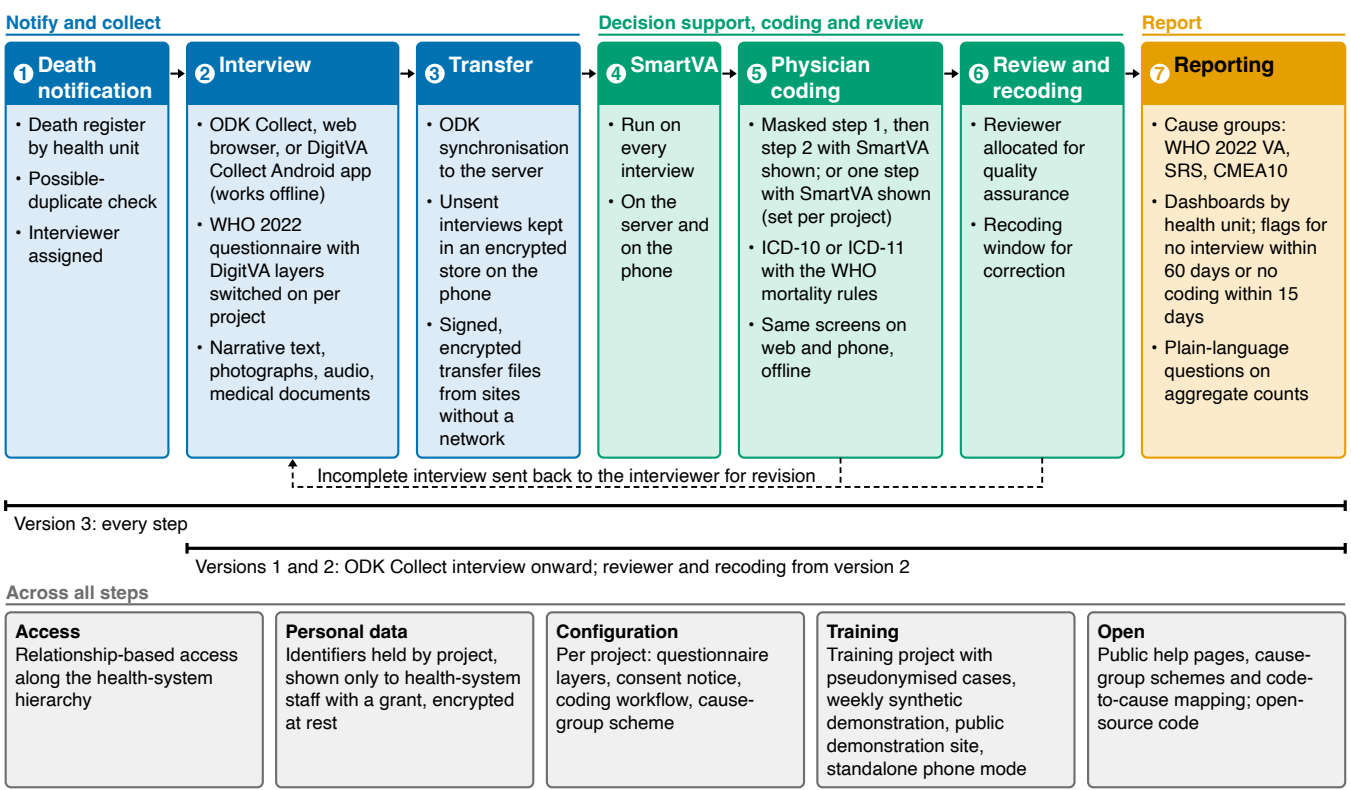
Notify and collect
Decision support, coding and review
Report
1 Death notification
• Death register by health unit
• Possible-duplicate check
• Interviewer assigned
2 Interview
• ODK Collect, web browser, or DigitVA Collect Android app (works offline)
• WHO 2022 questionnaire with DigitVA layers switched on per project
• Narrative text, photographs, audio, medical documents
3 Transfer
• ODK synchronisation to the server
• Unsent interviews kept in an encrypted store on the phone
• Signed, encrypted transfer files from sites without a network
4 SmartVA
• Run on every interview
• On the server and on the phone
5 Physician coding
• Masked step 1, then step 2 with SmartVA shown; or one step with SmartVA shown (set per project)
• ICD-10 or ICD-11 with the WHO mortality rules
• Same screens on web and phone, offline
6 Review and recoding
• Reviewer allocated for quality assurance
• Recoding window for correction
7 Reporting
• Cause groups: WHO 2022 VA, SRS, CMEA10
• Dashboards by health unit; flags for no interview within 60 days or no coding within 15 days
• Plain-language questions on aggregate counts
Incomplete interview sent back to the interviewer for revision
Version 3: every step
Versions 1 and 2: ODK Collect interview onward; reviewer and recoding from version 2
Across all steps
Access
Relationship-based access along the health-system hierarchy
Personal data
Identifiers held by project, shown only to health-system staff with a grant, encrypted at rest
Configuration
Per project: questionnaire layers, consent notice, coding workflow, cause-group scheme
Training
Training project with pseudonymised cases, weekly synthetic demonstration, public demonstration site, standalone phone mode
Open
Public help pages, cause-group schemes and code-to-cause mapping; open-source code